# Surgery for vulval cancer

You may have surgery to treat vulval cancer. The surgery you have will depend on the size and position of the cancer.

**On this page**


- About vulval cancer surgery
- Types of vulval surgery
- Reconstructing the vulva
- Pelvic exenteration
- How we can help


## About vulval cancer surgery

The aim of surgery for vulval cancer is to remove all the cancer. The surgeon will remove the area of skin where the cancer is and some healthy tissue all around it. How much tissue they need to remove depends on different things. This includes the size of the cancer, and where in the vulva it is. They may also remove some lymph nodes during the operation (explained under 'Removing and checking the lymph nodes in the groin' below).

### What to expect before surgery

Before your operation your surgeon will talk with you about the surgery and what it will involve. They will explain whether you are likely to need further operations to reconstruct the vulva. A specialist nurse will also give you information and support before and after your operation.

It can help to write down any questions you may have about the surgery before your appointment so you can talk through any worries.

### What to expect after surgery

Your surgeon or specialist nurse will also explain what to expect after surgery and about possible side effects. It is important to understand the possible changes and the support your team can give you to help you cope.

## Types of vulval surgery

There are different operations to treat cancer of the vulva, depending on the size and position of the cancer. If the cancer is very small, it may be possible to remove only a small area of the vulva.

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If the cancer is larger, you may need a bigger operation to remove more of the vulva. Many people may also need some lymph nodes removed during surgery.

## Wide local excision

This operation is sometimes called a radical local excision. The surgeon takes away the cancer and a border of healthy tissue all around the cancer. This is called the margin. The margin is usually at least 1cm wide. This helps to lower the risk of the cancer coming back.

If there are other vulval skin changes in the surrounding area, the surgeon usually removes this during surgery as well – for example, this may include vulval intraepithelial neoplasia (VIN).

You may have lymph nodes removed from one or both sides of the groin as well. This depends on where the cancer is, its size and how deeply it has grown.

### Radical wide local excision

*Image: The illustration shows the vulva, and the lymph nodes that are at the top of the legs in the groin area. There is a dotted line around part of the outer labia majora on the left-hand side to show the part of the vulva that is removed during a radical wide local excision. There is also a dotted line around the lymph nodes on the left-hand side to show that these are also removed.*

## Partial vulvectomy

If the tumour is larger, more of the vulval skin will be removed. This is to make sure there is a border of healthy tissue all around the cancer. This is called the margin. This helps to lower the risk of the cancer coming back.

This operation is sometimes called a radical partial vulvectomy.

You may also have lymph nodes removed from one or both sides of the groin. This may depend on where the cancer is, its size and how deeply it has grown.

Surgery to remove the front area of the vulva is called an anterior vulvectomy. This operation sometimes involves removing part of the clitoris or the end of the urethra – this is the tube that drains urine (pee) from the bladder. Removing part of the urethra does not usually affect how you pee. But some people find that it affects the spray of the pee.

Surgery to remove the back area of the vulva is called a posterior vulvectomy.

When only one side of the vulva is removed, it is called a hemi-vulvectomy.

The following diagrams show different types of radical partial vulvectomy.

### Radical partial vulvectomy – upper

*Image: The illustration shows the vulva, and the lymph nodes that are at the top of the legs in the groin area. There is a dotted line around the area that is removed during a radical partial vulvectomy for a cancer on the upper part of the vulva. This area includes the upper areas of the labia majora and labia minora. There is also a dotted line around the lymph nodes on both the left- and right-hand sides, showing that these will also be removed during this operation.*

### Radical partial vulvectomy – lower

*Image: The illustration shows the vulva, and the lymph nodes that are at the top of the legs in the groin area. There is a dotted line around the area that is removed during a radical partial vulvectomy for a cancer on the lower part of the vulva. This area includes the lower areas of the outer labia majora and inner labia minora, and the perineum. There is also a dotted line around the lymph nodes on both the left- and right-hand sides, indicating that these will also be removed during this operation.*

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### Radical partial vulvectomy – side

*Image: The illustration shows the vulva, and the lymph nodes that are at the top of the legs in the groin area. There is a dotted line around the area that is removed by a radical partial vulvectomy for cancer on the side of the vulva. The illustration uses the left side of the vulva as an example. There is a dotted line around the left-hand inner labia minora and outer labia majora, and the left side of the perineum. There is also a dotted line around the lymph nodes on the left side, showing that these will also be removed in this operation.*

## Vulvectomy

This operation removes the whole vulva, including the inner and outer labia, and the clitoris. It is also called a radical or total vulvectomy. This type of surgery may be needed if the cancer is:

- very large
- in the middle part of the vulva
- affecting lots of areas of the vulva, and with other vulval skin changes covering a large area.

The lymph nodes are also removed from one or both sides of the groin.

Your surgeon and specialist nurse can explain more about the type of surgery you might have. They will talk through what to expect and what support is available during recovery.

### Radical vulvectomy

*Image: The illustration shows the vulva, and the lymph nodes that are at the top of the legs in the groin area. There is a dotted line around the area that is removed during a radical vulvectomy. This area includes all the vulva, so the dotted line goes around all of the outer labia majora. There is also a dotted line around the lymph nodes on both the left and right sides, showing that these will also be removed during this operation.*

## Removing and checking the lymph nodes in the groin

Your surgeon or specialist nurse may talk to you about removing lymph nodes in the groin. They will explain what to expect and about any possible side effects.

During the operation, the surgeon may also remove:

- some nearby lymph nodes to check them for cancer cells – this is called a sentinel lymph node biopsy (SLNB)
- lymph nodes affected by cancer from one or both sides of the groin
- lymph nodes that are likely to contain cancer cells from one or both sides of the groin.

# Reconstructing the vulva

Depending on the area that is removed, you may need surgery to reconstruct the shape and form of the vulva. This is usually at the same time as the operation to remove the cancer. But it can also be done later.

If only a small amount of skin is removed from the vulva, you may not need reconstruction. The surgeon may be able to stitch the remaining skin neatly together. Or they may leave it to heal naturally. This will also depend on the site of the cancer.

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If a larger area of skin is removed, you may need to have a reconstruction with a skin graft or skin flap.

### Skin grafts

The surgeon takes a piece of skin from another part of the body and places it over the area where the cancer was removed from.

The place where the surgeon takes the skin from is called the donor site. This is usually the thigh or buttock. The place where the surgeon moves the skin to is called the grafted area. The amount of skin they take depends on the area they need to cover. Your surgeon will tell you more about this.

### Skin flaps

A skin flap is a slightly thicker layer of skin than a graft. The surgeon takes it from an area close to the vulva. They cut the flap away but leave it partially connected so it still has a blood supply. The surgeon moves the flap over the wound and stitches it in place. If you have a skin flap, you may need to stay in hospital for a few days.

Skin flap surgery is very specialised. You may have to travel to a different hospital to have it. If you need a skin flap, your surgeon will tell you more about it.

## Pelvic exenteration

Pelvic exenteration is a major operation where any affected organs are removed. It may be offered if the cancer has spread to organs close to the vulva. This includes the:

- womb
- bladder
- lower bowel.

It may still be possible to have an operation to remove the cancer.

Recovery after this type of operation can be difficult both physically and emotionally. So it is not done very often. But sometimes this operation may be the only way to completely remove the cancer.

This type of surgery is rare for vulval cancer. This is because it is more common to have chemotherapy and radiotherapy together. This is called chemoradiation. You doctor or nurse can give you more information.

## About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

### References

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Below is a sample of the sources used in our vulval cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Morrison J, Baldwin P, Hanna L, Andreou A, Buckley L, Durrant L, et al. British Gynaecological Cancer Society (BGCS) vulval cancer guidelines: An update on recommendations for practice 2023. European Journal of Obstetrics and Gynaecology and Reproductive Biology. 2024;292: 210–238. Available from: www.doi.org/10.1016/j.ejogrb.2023.11.013 [accessed July 2026].

The Royal College of Radiographers. The Royal College of Radiologists expert panel recommendations for radiotherapy treatment for vulval cancer. 2024. Available from: www.rcr.ac.uk/media/l1zjohkm/rcr-vulval-cancer-treatment-recommendations.pdf [accessed July 2026]

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Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

## The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
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structure the information clearly

- make sure important points are clear.

We use gender-inclusive language and talk to our readers as ‘you’ so that everyone feels included. Where clinically necessary we use the terms ‘men’ and ‘women’ or ‘male’ and ‘female’. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information here.

# How we can help

## Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.

### Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

### Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

### Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

### Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.

### Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help

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